

Annual Update and New Guideline Development Adult Telehealth Triage Guidelines



The Schmitt-Thompson telehealth triage guidelines are reviewed, updated, and then annually sent out to our clinical and software partners. This update process ensures that our guidelines are current, evidence-based, and compliant with the latest recommendations from national organizations.

Our triage guidelines are rigorously reviewed by physicians and nurses with experience in telehealth triage. Complex topic areas are also reviewed by clinical consultants and subject matter experts. Our Review Panel is comprised of triage nurses, specialty nurses, call center medical directors, primary care physicians, emergency medicine physicians and physician specialists.



The 3 R's of Clinical Content Development

The physician authors and the Review Panel follow the three R's of clinical content development.

- | | |
|-------------------|---|
| Reviewed | The guidelines have been reviewed by over 200 clinically active physicians in primary care and emergency medicine, physician specialists, call center nurse managers, and call center medical directors. All new guidelines and any guidelines with significant changes are subjected to and benefit from the combined review and input of the Schmitt-Thompson Clinical Content (STCC) editorial teams and external clinician experts. |
| Referenced | The guidelines are updated yearly and referenced to the current medical literature. |
| Researched | Published, peer-reviewed articles show consistent quality of call management using the STCC guidelines. Call center process and quality improvement outcome studies (unpublished research) are also used to update the clinical content annually. |

The Quadruple Aim

The *Triple Aim* is a framework developed by the Institute for Healthcare Improvement that describes an approach to optimizing health system performance.^{1 2} The three dimensions of the *Triple Aim* are:

1. Improving the patient experience of care (including quality and satisfaction)
2. Improving the health outcomes of populations
3. Reducing the per capita cost of health care.

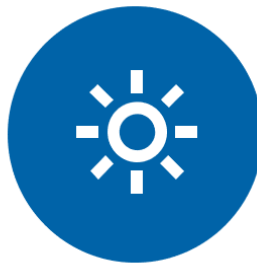
The STCC triage guidelines are developed and updated in adherence to the *Quadruple Aim*.³

While the *Triple Aim* focuses on patients, the *Quadruple Aim* adds the goal of improving the work lives of clinical staff. STCC triage guidelines are designed to serve as a decision support tool for telehealth nurses, providing helpful guidance and health information to improve nurse efficiency and confidence during telehealth patient encounters.



Better Patient Experience

Improving the patient experience of care (including quality and satisfaction).



Lower Costs

Reducing the per capita cost of health care.



Better Outcomes

Improving the health of populations.



Better Clinician Experience

Improving the work lives and performance of triage nurses.

¹ Berwick DM, Nolan TW, Whittington J. The triple aim: care, health, and cost. *Health Aff (Millwood)*. 2008;27(3):759-769.

² Triple Aim. IHI. <http://www.ihl.org/engage/initiatives/TripleAim/Pages/default.aspx>

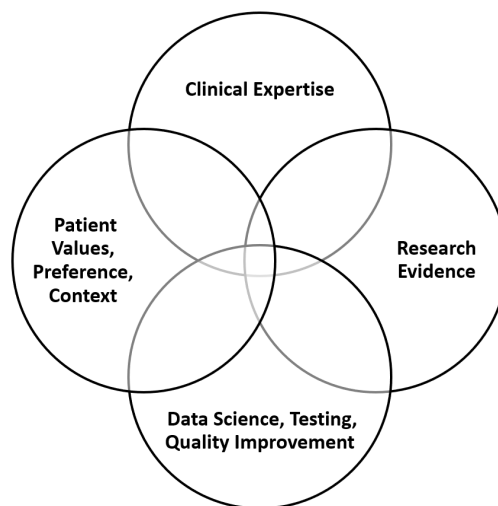
³ Bodenheimer T, Sinsky C. From triple to quadruple aim: care of the patient requires care of the provider. *Ann Fam Med*. 2014;12(6):573-576.

Levels of Evidence

STCC strives to use an evidence-based approach for developing and updating the clinical content.

The STCC Clinical Editorial Team utilizes a standardized hierarchy for evaluating the quality of the evidence and deciding how to best apply this information in a practical manner.

Evaluating research quality is a challenging task. Often published research-based evidence is lacking or missing. It is important to “balance evidence-based practice with practice-based medicine,” while considering patient preferences.



Results from data science, testing, and quality improvement activities should be an integrated part of clinical content development activities.

Annual Update

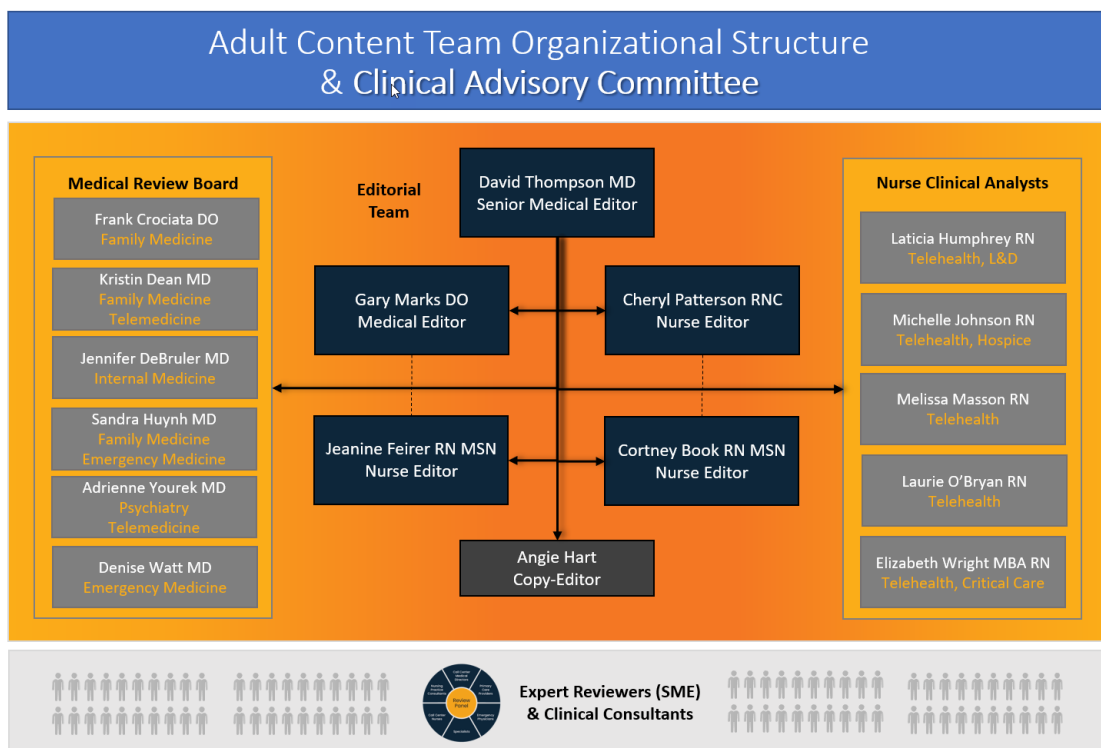
The Schmitt-Thompson clinical content has an ongoing quality assurance program that results in continual improvement.

Every year we work to improve the existing guidelines with input from our clinical partners, usage data, medical evidence (literature review), and feedback from a broad panel of reviewers and subject matter experts. Additions and improvements that are included in an **annual update** include the following data sources:



- **User feedback** from our call center and clinical partners is a core element in the continued improvement of the STCC telehealth triage guidelines. Many call centers in the United States and Canada provide STCC with quarterly or annual clinical feedback. We also receive other suggestions and questions on a nearly daily basis. As part of our client support and continuous quality improvement process, we provide a detailed written response for all feedback that we receive. We are especially proud of this part of our update process. Call center input is a rich and valuable source of change and improvement. The universal changes we make to the guidelines benefit all of our partners and the patients they serve.

- Formal **quality improvement** or outcome study data from call centers. Here are 3 examples of **call audits** from call centers partners:
 - a. Chart Documentation or Audio Review of calls with the same presenting chief complaint.
 - b. Chart Documentation or Audio Review of calls with the same disposition.
 - c. Chart Documentation or Audio Review of calls with the same triage question.
- Planned and scheduled ongoing **medical literature review** for articles that are relevant to existing guidelines. Clinical research findings and recommendations that improve the guideline content are incorporated into the decision-logic, care advice, and health information. Articles that expand and support the evidence base are logged and added to the clinical content database. STCC includes all these research citations in the database that we distribute to our clinical and software partners.
- **Quantitative analysis** of guideline utilization and triage disposition rates for a sample of over 6 million calls from 13 medical call centers and other data from our software partners.
- Changes in **national guidelines** of the AAP, AAFP, ACEP, ACOG, CDC, FDA, ISDA and other national organizations.
- Input from our **Clinical Advisory Committee** and other expert of our Review Panel.

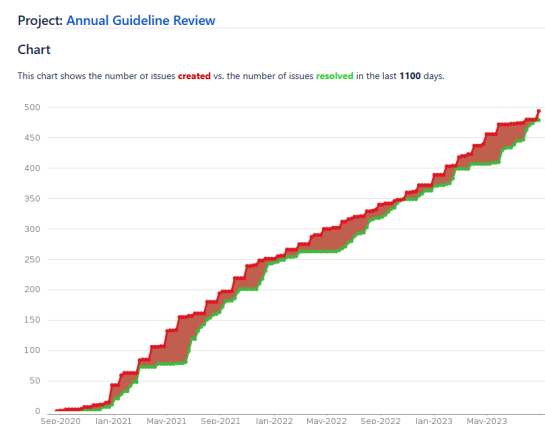
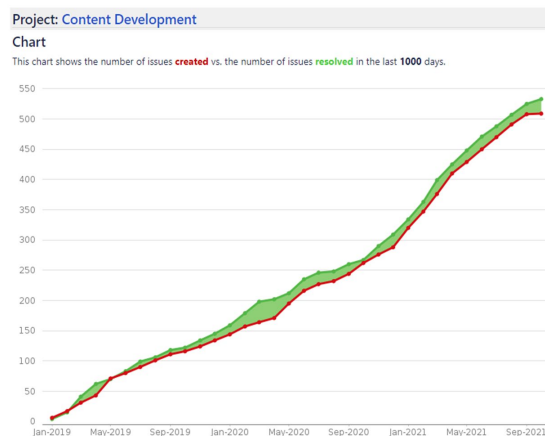


Using these many data sources, we review and update all affected guidelines prior to publishing the annual update. Changes are tracked following our clinical governance policies and procedures. Review and changes are performed primarily by our Editorial Team to ensure consistency and parallel logic across related guidelines. Broader review is obtained from our Clinical Advisory Committee, Specialty Work Groups, and other clinical consultants and subject matter experts if changes are substantive or complex.

In addition, and regardless of interval reviews, all of the telehealth triage guidelines are formally reviewed by two telehealth triage nurses and one physician on a 3-year cycle. Again, broader review is obtained from our Clinical Advisory Committee, Specialty Work Groups, and other clinical consultants and subject matter experts if changes are substantive or complex.

Included in every annual update are redline comparisons (to the prior year) of all guidelines to clearly show our clinical partners exactly what minor and major changes have been made each year.

We also write a detailed annual letter to triage nurses, detailing a summary of changes. This serves as a self-study guide for the updated telehealth triage clinical content.



Ankle Swelling
After Hours Telehealth Triage Guidelines | Adult | 2022-2023

Schmitt-Thompson Clinical Content

DEFINITION

- Swelling up or around an ankle joint.
- Swelling up or around an ankle joint that is different from the other ankle.
- Swelling up or around an ankle joint that is a traumatic injury.

TRIGGER ASSESSMENT QUESTIONS

See More Appropriate Outcomes

Chest Pain

Go to Guideline: Chest Pain (Adult)

Followed an ankle injury

Go to Guideline: Ankle and Foot Injury (Adult)

TRIGGER ASSESSMENT QUESTIONS

Swelling of both ankles (i.e., pedal edema)

Go to Guideline: Leg Swelling and Edema (Adult)

Swelling of calf or heel

Go to Guideline: Leg Swelling and Edema (Adult)

Small areas of redness or swelling followed on-month into the area

Go to Guideline: Leg Swelling and Edema (Adult)

Schmitt-Thompson Clinical Content

Adult After-Hours Telehealth Triage Guidelines

Update Letter 2022 – Call Center Triage Nurses

April 27, 2023

Dear Telehealth Triage Nurse Colleagues:

Yearly updates and new topics bring with them the responsibility to read and study significant or major changes. Trying to learn new material while managing an actual call can be difficult.

We hope this summary of changes will serve as a self-study guide, direct your reading, and help you transition to the 2023 Adult After Hours (A/H) Telehealth Triage Clinical Content.

New Guidelines

There are 20 new adult guidelines since the last annual update in 2022.

We are adding 7 hospice telehealth triage guidelines. These along with the updated existing hospice guidelines gives us a count of 20 in this specialty area.

- Hospice - Cough/Need for Support or Rescue
- Hospice - Medication Question or Refill Call
- Hospice - Oral Intake Decreased
- Hospice - Seizure
- Hospice - Skin Breakdown and Pressure Sores
- Hospice - Swallowing Difficulty
- Hospice - Urinary Symptoms

In addition to these we have 13 other new adult A/H telehealth triage guidelines.

- Abdomen Swelling and Swelling
- Acid
- Bull (Skin Abscess) on Treatment Follow-up Call
- Cancer - Breathing Difficulty
- Coughed or Dry Cough
- Menopausal Symptoms and Questions
- Nausea - Chiropractic or Acupuncture (HBM release in July 2022)
- Nausea - Excessive (Initial release in July 2022)
- Pregnancy - High Blood Pressure
- Pregnancy - High Blood Pressure
- Pregnancy - Nausea
- Weight Loss - Unintended
- Wound Infection on Antibiotic Follow-up Call

Schmitt-Thompson Clinical Content

Page 1 of 42

New Guideline Development

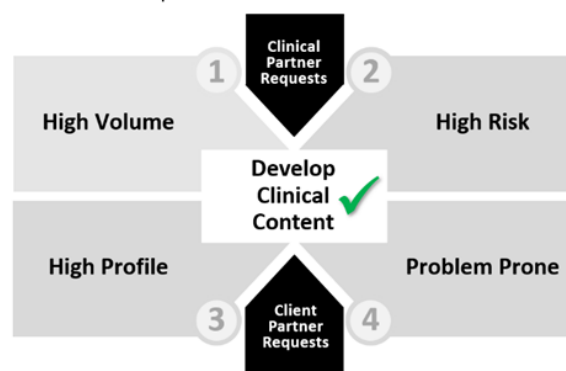
New guideline development follows an 11-step process.

1	Review related guidelines and information submitted by requesting call centers.	
2	Write First Draft based on guideline review, clinical experience, and STCC guideline templates.	
3	Research guideline using <i>Levels of Evidence</i> policy and input from subject matter experts.	
4	Construct differential diagnosis. Identify high frequency and high-risk causes.	
5	Write Second Draft .	
6	Review and editing by Senior Medical Editor (Third Draft).	
7	Review of guideline by Medical Review Board, external Review Panel, and subject matter experts.	
8	Write Fourth Draft .	
9	Testing and proofing by Copy-Editor.	
10	Write Final Version .	
11	Approval by Senior Medical Editor	

How is future triage content development prioritized? Input from our call center partner customers drives the development decisions. There are four patient-focused **reason-for-call (RFC)** factors that are considered.

1. High Volume
2. High Risk
3. High Profile
4. Problem Prone

What is the STCC framework for prioritizing NEW clinical content development?



We welcome suggestions for future guidelines.

Our Mission

To provide the highest quality clinical content for telehealth triage in the world.

To update and expand the clinical content on an annual basis, based upon recent medical literature, recent national guidelines, quality improvement projects, research, expert reviews and feedback from users.

To provide ongoing support to providers, nurse triagers, and call centers that use our content.